

HOW TO OPEN A NON-MEDICAL HOME CARE AGENCY in Virginia

A VDH-licensed Home Care Organization — private pay & Medicaid (Cardinal Care).

A complete, plain-language, step-by-step startup guide — written so you can actually DO each step.

Figures current for 2026 · Every contact, rate & fee should be re-verified before you rely on it.

The Virginia model — read this first

In Virginia, a business that provides hands-on personal care in people's homes must be licensed as a Home Care Organization (HCO) by the Virginia Department of Health (VDH), Office of Licensure and Certification, under Code of Virginia Title 32.1 and regulations 12VAC5-381. One Virginia rule shapes your whole plan: if you provide ONLY companion, sitter, homemaker, or chore services (no hands-on personal care), you're EXEMPT from licensure — but the moment you add personal care (bathing, dressing, transferring, toileting, feeding), you need the HCO license. This guide covers the licensed HCO path (the one that lets you do personal care and bill Medicaid), and flags the companion-only exemption. It's broken down so a first-timer can follow it.

The Virginia line — companion-only is exempt; personal care is licensed. Under 12VAC5-381-30, licensure does NOT apply to providers of ONLY homemaker, chore, or companion services. So a pure companion/sitter/homemaker business (companionship, transportation, meal prep, light housekeeping, shopping — no hands-on ADL care) can operate WITHOUT a VDH license. But if you provide personal care — hands-on help with bathing, dressing, transferring, toileting, or feeding — you MUST be a licensed Home Care Organization. Most agencies that want a real elder-care market (or to bill Medicaid) provide personal care and need the license. This guide is that path.

Accreditation — optional. Virginia does NOT require national accreditation for an HCO license. Accreditation (ACHC, CHAP, or The Joint Commission) is often preferred by VA and managed-care contracts and helps win referral partners. Every PolicyReady product is built to those standards behind the scenes; treat accreditation as a goal for year two (Part 9).

How to use this guide

Read it once start to finish, then use it as a checklist and a phone list. It's written for someone who has never opened an agency — where we name a thing, we break it down and tell you exactly where to go and what to ask. It is general information, not legal, tax, or clinical advice; verify each step with VDH, DMAS, and qualified counsel (see Part 11) before you rely on it.

The journey at a glance

1	Decide this is your business	A licensed home care agency (or companion-only) — recurring revenue (Parts 1–2)
2	Form the business & insure	Entity, EIN, NPI, and the right insurance & bonding (Part 3)
3	Get the VDH license	Application, the \$500 fee, the pre-licensure survey (Part 4)
4	Build your leadership	An administrator + an RN/LPN available during care (Parts 4–5)
5	Recruit & screen aides	Aide qualifications, VSP check + sworn disclosure, TB (Part 5)
6	Set up EVV & operations	EVV, policies, systems (Part 6)

7	Serve private-pay clients	Start earning while Medicaid enrollment proceeds (Part 8)
8	Enroll for Medicaid (Cardinal Care)	DMAS/Gainwell enrollment + the MCOs (Part 8)
9	Get your first clients	Discharge planners, the 25 AAAs, No Wrong Door (Part 10)
10	Who to call & what to ask	Every contact + the exact questions, in one place (Part 11)

Rule of thumb. Virginia requires a pre-licensure on-site survey, so build your policies and files to the 12VAC5-381 standard before you apply. Line up your RN/LPN coverage early (required during personal-care hours), and launch on private pay while Medicaid enrollment proceeds.

PART 1

What This Business Is

In short: A VDH-licensed Home Care Organization providing personal care in clients' homes — hands-on help with daily living. Agency-based, paid by private pay and Medicaid.

Virginia licenses Home Care Organizations to provide an organized program of home health, pharmaceutical, or personal care services in a client's residence (12VAC5-381-10). A non-medical agency licenses as an HCO providing personal care — hands-on help with the activities of daily living (bathing, dressing, grooming, toileting, transferring, feeding, mobility). Companion, homemaker, and chore services are defined separately and, on their own, are exempt from licensure.

Personal care vs. companion-only

	Personal care (licensed HCO — this guide)	Companion-only (exempt)
What it is	Hands-on ADL help	Companionship, homemaker, chore, sitter
License	VDH Home Care Organization license required	No VDH license (12VAC5-381-30 exemption)
Medicaid	Can bill Cardinal Care personal care	Generally not a Medicaid personal-care provider
Supervision	RN/LPN available during care	No clinical supervision requirement

Key terms. VDH = Virginia Dept. of Health (your licensor). OLC = Office of Licensure and Certification. HCO = Home Care Organization. 12VAC5-381 = the HCO regulations. DMAS = Dept. of Medical Assistance Services (Medicaid). Cardinal Care = Virginia Medicaid's brand. CCC Plus waiver = the long-term-care waiver. MCO = a Cardinal Care managed-care plan. VSP = Virginia State Police (background checks). EVV = Electronic Visit Verification. No Wrong Door / VirginiaNavigator = the aging referral network. NPI = National Provider Identifier.

Why this business — and why now

Demand for in-home care is rising fast, and for reasons that aren't going to reverse: people overwhelmingly want to age in their own homes rather than move to a facility; the 65-and-over population grows every year; and hospitals now discharge patients "quicker and sicker," which sends more families looking for help at home right after a hospital stay. Virginia's 65-and-over population is among the fastest-growing segments in the Commonwealth, and the mix of dense Northern Virginia suburbs and aging rural communities creates steady, recurring demand for in-home care statewide. And it's a business you can start lean — there's no building to buy or fill, the revenue is recurring and hourly, and a good reputation compounds through referrals.

The three mistakes that sink new agencies — avoid these. New home-care owners rarely fail because of the care itself. They fail for three business reasons: (1) undercapitalization — they run out of cash covering weekly payroll before Medicaid or insurers reimburse (see the cash-flow note in Part 9); (2) weak caregiver recruiting and retention — they can't reliably staff the shifts they're offered, so referral sources quietly stop calling; and (3) sloppy compliance — a missing background check, training record, or (for Medicaid) EVV visit that triggers a payback or a survey finding. Get your cash runway, your staffing pipeline, and your paperwork right, and the rest follows.

PART 2

Who Regulates You (and What Each One Does)

In short: VDH licenses and surveys you, DMAS/Gainwell and the Cardinal Care MCOs pay you for Medicaid, the Virginia State Police screen your staff, and EVV verifies Medicaid visits.

VDH — Office of Licensure and Certification (your licensor)

The Virginia Department of Health's OLC issues your HCO license, writes the 12VAC5-381 standards, reviews your application, and conducts the pre-licensure conference and on-site survey. OLC general line (804) 367-2102; applications through the VDH OLC Portal; vdh.virginia.gov/licensure-and-certification.

DMAS, Gainwell & the Cardinal Care MCOs (Medicaid)

Virginia Medicaid (Cardinal Care), administered by DMAS, covers personal care, respite, and companion services through the CCC Plus waiver, delivered by managed-care plans. You enroll as a Medicaid provider through Gainwell's PRSS portal and contract with each Cardinal Care MCO.

Virginia State Police (background checks)

Before employment, each aide must have a Virginia State Police criminal record report and complete a sworn disclosure statement; no one convicted of a barrier crime (§ 32.1-162.9:1) may be employed.

EVV — Electronic Visit Verification

For Medicaid personal care, respite, and companion services (agency- and consumer-directed), Virginia requires Electronic Visit Verification. A visit must be verified to be paid. Private-pay work isn't subject to EVV.

What this means for you: several different bodies touch your business — VDH/OLC (the license and the survey), the Virginia State Police (your aides' background checks), DMAS with Gainwell and the Cardinal Care MCOs (your Medicaid pay), and the EVV system (which verifies every Medicaid visit). It sounds like a lot, but each has a narrow job, and this guide tells you exactly who to call for what (Part 11). Keep a simple one-page contact sheet on your wall from day one, and you'll never wonder who owns a given question.

PART 3

Form Your Business & Insure — Step by Step

In short: *This is where most first-timers get stuck. We break each piece all the way down — what it is, where to go, exactly what to do, and what to have in hand.*

3.1 Form your legal entity (an LLC)

An “entity” is the legal business — almost always a Limited Liability Company (LLC) — that holds your license or registration, your contracts, and your bank account, and that shields your personal assets if the business is ever sued. In Virginia you form the LLC through the State Corporation Commission (SCC), filing Articles of Organization online at the Clerk's Information System (cis.scc.virginia.gov) for a \$100 filing fee. You'll name a “registered agent” (a person or service with a physical in-state address who receives legal mail — this can be you) and adopt an Operating Agreement (the internal document that sets out who owns what and who can make decisions).

- 1. File your Articles of Organization (Form LLC-1011).** Through the SCC Clerk's Information System at cis.scc.virginia.gov — the filing fee is \$100. Name your LLC and list a Virginia registered agent.
- 2. Name a registered agent and adopt an Operating Agreement.** The agent (which can be you) receives legal mail at a Virginia address; the Operating Agreement sets out ownership, management, and how money is handled.
- 3. Get your local business license (BPOL).** Most Virginia localities require a Business, Professional & Occupational License from your city or county before you operate — check with your local commissioner of the revenue.
- 4. Calendar your annual registration fee.** Virginia LLCs pay a \$50 annual registration fee to the SCC each year to stay in good standing.

No newspaper publication in Virginia. Unlike New York, Virginia has no LLC publication requirement — forming the entity is a straightforward SCC filing. Your remaining setup pieces are the EIN, the NPI, the business bank account, and the local BPOL license, covered below.

3.2 Get your EIN — the free federal tax ID

An EIN (Employer Identification Number) is your business's federal tax number — think of it as a Social Security number for the company. It's free, takes about ten minutes, and you need it before you can open a bank account, run payroll, or enroll with any payer. Here's exactly how to get it:

- 1. Go to [IRS.gov](https://irs.gov) and open the EIN Assistant.** Search “Apply for an EIN online.” Apply directly on the IRS website — never pay a third-party site that charges for this free service.
- 2. Choose your entity and enter the responsible party.** Select “Limited Liability Company,” then give the responsible party's legal name and SSN or ITIN (usually you, the owner), plus your business name and address.
- 3. Finish in one sitting.** The online session times out after about 15 minutes of inactivity and can't be saved partway — have your details ready before you start.
- 4. Save your EIN and the CP-575 letter.** You get the EIN on the spot and can download the official CP-575 confirmation letter — save both; your bank and every payer will ask for them.

3.3 Get your NPI — the national provider ID

An NPI (National Provider Identifier) is a free, ten-digit ID number that health-care payers use to identify your organization. You'll use it to enroll with Virginia Medicaid (Cardinal Care) through DMAS and bill the managed-care plans. Your AGENCY needs a Type 2 (Organizational) NPI — which is different from the Type 1 (individual) NPI a single person would get.

- 1. Create an I&A account.** At nppes.cms.hhs.gov, set up an Identity & Access (I&A) account for whoever will manage the number — usually an owner.

2. Start a Type 2 (Organizational) application. Log into NPPES and choose Type 2, not Type 1. Type 1 is for an individual person; your business is a Type 2 organization.

3. Enter your legal name, EIN, and address, and pick your taxonomy. Choose the taxonomy that fits a home-care / personal-care agency. The “taxonomy” is the code that says what kind of provider you are — confirm the exact one your Medicaid program wants before you submit.

4. Submit — it's free and quick. It usually processes in about 10 business days. Save the NPI; you'll use it on every enrollment and claim.

3.4 Open a business bank account

Keep the business's money completely separate from your personal money — it protects your liability shield and makes payroll, taxes, and payer deposits clean. Open the account once your entity and EIN are set up.

What to bring to the bank:

- Your EIN confirmation (CP-575) from the IRS
- Your filed Articles of Organization / Certificate of Formation and the entity ID number
- Your signed Operating Agreement
- A Beneficial Ownership form (the bank collects details on anyone owning 25%+ plus a control person)
- A government photo ID, SSN, date of birth, and home address for each owner/signer
- An opening deposit (often around \$100)

Where to go: a bank with lots of Virginia branches — Truist, Bank of America, Wells Fargo, Atlantic Union (a Virginia-based bank), or a local Virginia credit union. Home care is payroll-heavy — you pay caregivers weekly, but Medicaid and insurers pay you weeks later — so ask specifically about a business line of credit or invoice financing to bridge payroll before you scale.

3.5 Get your insurance — what you need, who sells it, and what to ask

A home-care agency sends employees into clients' homes, so you need coverage built for that specific risk:

Coverage	What it protects against	Must-have?
General & Professional Liability	A slip-and-fall or property damage in a client's home, plus claims about the CARE (an aide's error or neglect)	Yes — foundational
Abuse & Molestation	Allegations of abuse by an aide — general liability EXCLUDES this	Yes — high exposure
Employee Dishonesty / Surety Bond	Theft from a client's home — families and payers expect caregivers to be “bonded”	Yes — clients & payers expect it
Non-Owned & Hired Auto	Your aides driving their own cars to clients or with clients	Yes — critical for home care
Workers' Compensation	Aide injuries (lifting and transferring are the common ones)	Required in Virginia for employees

Real places to get it

Buy through an independent agent or broker who writes home-care programs — carriers and programs active in this niche include Philadelphia Insurance (PHLY), CNA, The Hartford, Markel, and home-care specialty programs from Caitlin Morgan or NIP Group / HomeCareUSA. To find a local Virginia agent, use [trustedchoice.com](https://www.trustedchoice.com) (filter for home care). Confirm the carrier writes non-medical home care in Virginia before you rely on a quote.

What to ask the insurance agent

“Do you write Virginia Home Care Organizations — which carriers, and what's the minimum premium?”

“Is abuse & molestation included or a sub-limit, and do you include non-owned & hired auto (my aides drive their own cars)?”

“Can you place an employee-dishonesty bond so I can tell families my caregivers are bonded?”

“What coverage limits do the Cardinal Care MCOs, the VA, or long-term-care insurers require?”

“What will Virginia workers' compensation cost given my aide payroll?”

PART 4

Get Your VDH License — Step by Step

In short: Assemble your application and policies, pay the \$500 fee, and pass the pre-licensure conference and on-site survey. VDH targets about 60 days for a complete application.

4.1 Before you apply

- ☐ Entity formed; EIN, NPI, and local business license (BPOL) obtained; insurance & bond bound
- ☐ A designated administrator and an RN/LPN arrangement for personal-care hours (Part 5)
- ☐ Your policies & procedures written to 12VAC5-381 (client care, aide qualifications, background checks, supervision, complaints, incidents, emergency, infection control)

4.2 The application & survey

- 1. Apply through the VDH OLC Portal.** Submit your HCO application with ownership, services, and policies.
- 2. Pay the fee.** The HCO license fee is \$500, nonrefundable (confirm the current amount on the OLC Portal).
- 3. Attend the pre-licensure conference.** OLC schedules a conference and an on-site pre-licensure survey by Home Care Licensing staff.
- 4. Correct any deficiencies.** A deficiency report may be issued with a 15-day correction window.
- 5. Receive your license.** VDH targets about 60 days from a complete application to issuance.

4.3 Timeline

Plan for roughly two months from a complete application through the survey to your license, plus the time to write your policies to standard. Start your Medicaid (Gainwell) enrollment and aide hiring in parallel, and launch on private pay as soon as you're licensed.

PART 5

Your Administrator, RN & Aides — by Position

In short: *Virginia sets specific aide qualifications, requires nurse availability during personal-care hours, and mandates background checks. Here it is, one role at a time.*

5.1 Your administrator & nurse coverage

An HCO must have a designated administrator responsible for day-to-day operation. For personal care, an RN or LPN must be available during all hours personal-care services are provided, and a nurse makes supervisory visits as the client's needs require — not to exceed every 90 days. Line up your nurse coverage early (employed or contracted).

5.2 Your personal care aides (12VAC5-381-290)

- **Qualification pathways:** a home attendant providing personal care must meet one of several pathways — e.g., completing a Board of Nursing-approved nurse aide program (or holding CNA certification), passing a competency evaluation to the federal standard, or completing the state Personal Care Aide Training Curriculum. All aides must speak, read, and write English.
- **The “40-hour” rule:** the 40-hour PCA training requirement is a DMAS Medicaid rule for aides at non-VDH-licensed agencies; VDH-licensed HCOs satisfy training through the 12VAC5-381-290 pathways. If you'll bill Medicaid, meet the DMAS training standard too.
- **Ongoing training:** at least 12 documented hours of in-service training each year.

5.3 Background checks & health (12VAC5-381-110)

1. **Get the Virginia State Police criminal record report.** Within 30 days of employment (and not dated more than 90 days before employment).
2. **Complete the sworn disclosure statement.** Filed with the criminal record report; no one convicted of a barrier crime (§ 32.1-162.9:1) may be employed.
3. **Check any payer-required registries.** The HCO rule requires the VSP check + sworn disclosure; a central-registry (child/adult abuse) check may be required in certain Medicaid/child-care contexts — confirm per payer.
4. **Confirm TB and document everything.** Follow your infection-control/employee-health policy (confirm the current TB expectation with OLC) and keep dated results in each file.

5.4 The Virginia personal-care aide training & competency pipeline

The single question first-time Virginia owners get wrong is how their aides become qualified — because there are two overlapping standards, one from VDH (for the license) and one from DMAS (for Medicaid billing). Get both right and your aides can work any client; get them confused and you'll fail a survey or lose a Medicaid claim. Here is the pipeline, in order:

1. **Pick a qualification pathway for each aide (12VAC5-381-290).** A home attendant qualifies by one of several routes — completing a Virginia Board of Nursing-approved nurse aide education program (or holding an active CNA certificate), passing a competency evaluation to the federal home-health-aide standard, or completing the state Personal Care Aide (PCA) Training Curriculum. Document which pathway each aide used.
2. **Layer in the DMAS 40-hour standard if you'll bill Medicaid.** For Cardinal Care personal care, DMAS expects aides to meet its personal-care-aide training standard (commonly summarized as the 40-hour curriculum plus a competency check). A VDH-licensed HCO satisfies training through the 12VAC5-381-290 pathways, but confirm each aide also meets the DMAS standard before you assign them to a Medicaid client.
3. **Verify English literacy and run the competency check-off.** All aides must speak, read, and write English well enough to follow a plan of care and document. Do a hands-on skills check-off before an aide works alone — this is both a survey expectation and your best protection against a bad first visit.

4. Schedule the 12 hours of annual in-service. Every aide needs at least 12 documented hours of in-service training each year. Calendar it from each hire date and keep the certificates in the aide's file — surveyors pull these first.

The two-standard trap. VDH licensure and DMAS Medicaid billing each have their own training expectation. The safest move is to train every aide to BOTH from day one, so you never have to hold an aide back from a Medicaid client — and so a surveyor or MCO auditor always finds a complete, dated training record.

5.5 Recruiting and keeping good caregivers — the real make-or-break

Every experienced agency owner will tell you the same thing: your business rises or falls on caregivers, not clients. Referral sources will send you all the clients you can handle — but only if you can reliably staff them. So treat recruiting and retention as your core operation, not an afterthought.

- **Recruit constantly:** post continuously (Indeed, local Facebook groups, word of mouth, and a caregiver-referral bonus), and make applying fast — the best caregivers take the first agency that calls them back and schedules an interview.
- **Hire for reliability and warmth:** specific skills can be trained; showing up on time and being kind cannot. Check references for dependability, not just experience.
- **Onboard properly:** a real orientation, a skills check-off before a caregiver works alone, and a written scope of what they may and may not do — this protects clients and builds the caregiver's confidence.
- **Retain deliberately:** consistent hours, quick and correct pay, being reachable when a caregiver has a problem, genuine recognition, and a small raise ladder. Turnover is the biggest hidden cost in home care — every caregiver who quits costs you recruiting, training, and often the client they served.

PART 6

EVV, Policies & Getting Operational

In short: *Your policies come from the licensure standards; add EVV for Medicaid visits and your scheduling/billing systems, and keep files survey-ready.*

6.1 Your policies & procedures

Build your P&P Manual to 12VAC5-381: client rights, the plan of care and nurse supervision, aide qualifications and training, background checks, complaint handling, incident and abuse reporting, emergency preparedness, and infection control. This is what the surveyor reviews at your pre-licensure survey.

6.2 Set up EVV (before your first Medicaid visit)

Virginia requires Electronic Visit Verification for Medicaid personal care, respite, and companion services (agency- and consumer-directed). Onboard, set up your aides/clients/authorizations, capture every visit, and bill against verified visits. Questions: EVV@dmas.virginia.gov. Private-pay work isn't subject to EVV.

6.3 Systems

Pick a home-care platform for scheduling, records, visit tracking, invoicing, and payroll that supports Virginia EVV.

6.4 Documentation & staying inspection-ready

Whatever your state requires, the agencies that pass surveys and payer audits cleanly all do the same thing: they keep complete, current files from day one instead of scrambling the week before an inspection. Build these habits now, because reconstructing records after the fact is nearly impossible:

- **A file for every caregiver:** the application, background checks, training and competency records, health/TB screening, and a signed acknowledgment of your policies — all dated and kept current.
- **A file for every client:** the assessment, the service/care plan, the signed service agreement, consents, ongoing visit notes, and any payer authorizations.
- **A visit record for every shift:** who worked, when they arrived and left, and what care was provided — and, for Medicaid, the matching EVV record.
- **An incident and complaint log:** every incident, injury, and complaint and exactly how you resolved it — inspectors look for a real, used quality-improvement process, not a binder that's never been opened.

PART 7

The Client's Journey — Intake to Discharge

In short: A client is referred (privately or via a Cardinal Care MCO), your nurse assesses and sets the plan of care, an aide delivers the hours (with EVV for Medicaid), the nurse supervises, and you review and adjust.

Stage 1 — Referral & inquiry

A referral comes from a family, a hospital discharge planner, a home-health/hospice partner, or a Cardinal Care MCO care coordinator (Part 10).

What you do on the first call:

- Take the basics — who needs care, what tasks, how many hours, where, and when to start.
- Confirm who pays (private pay, Cardinal Care, VA, or long-term-care insurance) and that the need is within your scope (personal care vs. companion-only).

Stage 2 — Assessment & the plan of care

Your nurse assesses the client in the home and builds the plan of care. For Medicaid clients, the MCO's assessment and authorization set the approved hours; your plan of care aligns to it.

What to review in the home:

- Mobility and transfer needs; fall hazards; medications and who manages them; family and other supports.
- For Cardinal Care clients, the MCO's authorization — which sets the approved hours your plan of care and your EVV visits must match.

Stage 3 — Agreement & start of care

The client (or representative) signs a service agreement (services, schedule, rate/payer, rights, termination). Assign a qualified, screened aide and begin with nurse oversight.

Stage 4 — Delivering the hours (with EVV for Medicaid)

The aide delivers care; Medicaid visits are captured in EVV. Protect the client-aide match — consistency drives satisfaction and retention.

Stage 5 — Nurse supervision & quality

Your RN/LPN makes supervisory visits (not to exceed every 90 days); you monitor performance and changing needs, run your quality program, and report any suspected abuse, neglect, or exploitation.

Stage 6 — Changes & reassessment

As needs change, the nurse updates the plan of care; for MCO clients, coordinate reassessments and new authorizations. Keep authorizations current so delivered hours get paid.

Stage 7 — Discharge & transition

When care ends, follow your discharge policy, give notice, coordinate with the family and any MCO, and document. A good discharge turns families into referral sources.

Keep authorizations ahead of the care. The most common way Virginia agencies lose money isn't bad care — it's delivering hours that run past an expired MCO authorization. Track each Cardinal Care client's authorized units and reassessment date, and start the renewal before the current authorization runs out, so every delivered (and EVV-verified) hour is a paid hour.

PART 8

How You Get Paid — Private Pay, Medicaid (Cardinal Care), VA & LTC Insurance

In short: Start on private pay and long-term-care insurance the day you're licensed; add Medicaid by enrolling with DMAS and contracting with the Cardinal Care MCOs. Personal care is billed hourly.

Your four revenue streams:

Source	What it is	Key fact
Private pay	Client/family pays out of pocket, hourly	Start immediately; ~\$30/hr in VA (2024); no EVV/enrollment
Medicaid — Cardinal Care	CCC Plus waiver personal care/respite/companion	Through 5 MCOs; requires enrollment, contracts & EVV
Long-term-care insurance	The client's LTC policy reimburses in-home care	Bill the family or insurer once benefit triggers are met
Veterans (VA)	VA Homemaker/HHA, Veteran-Directed Care, Aid & Attendance	Serve veterans via the Community Care Network (Region 1, Optum)

8.1 Private pay — your launch market

The day your license issues, serve private-pay clients — families paying out of pocket, typically billed hourly with a visit minimum. Virginia private-pay non-medical care runs roughly \$30/hour (2024 cost-of-care data — verify the current figure). Set your rate above your fully-loaded aide cost. Long-term-care insurance is part of this stream: many policies reimburse in-home personal care once ADL/cognitive triggers are met (most require a licensed agency) — you bill the family or the insurer.

8.2 Medicaid — Cardinal Care

Virginia Medicaid (Cardinal Care) covers personal care, respite, and companion services through the CCC Plus waiver, delivered by five managed-care plans (SFY2026): Aetna Better Health of Virginia, Anthem HealthKeepers Plus, Humana Healthy Horizons, Sentara Health Plans, and UnitedHealthcare. Consumer-directed services (personal assistance, respite, companion) are also available, with fiscal agents that vary by plan (PPL and CDCN).

- 1. Hold your HCO license first (Part 4).**
- 2. Enroll as a Medicaid provider.** Through DMAS via Gainwell's PRSS portal (vamedicaid.dmas.virginia.gov) — updated enrollment requirements took effect July 1, 2025.
- 3. Contract with each Cardinal Care MCO** you want to serve; complete credentialing.
- 4. Set up EVV (Part 6)** before you deliver a Medicaid visit.

8.3 The rate & billing

On the schedule effective July 1, 2025, agency-directed personal care/respite/companion pays \$23.81/hour in Northern Virginia and \$20.23/hour in the rest of the state (consumer-directed is lower — \$17.97 NOVA / \$13.88 ROS). Verify the current figures with DMAS. Bill through the MCO against verified EVV visits, and confirm the timely-filing window.

How you actually bill (the claims cycle)

- 1. Verify eligibility & the authorization each month.** Confirm the member's Cardinal Care plan and the authorized hours before and during care — delivering past an expired authorization is the most common unpaid-claim mistake.
- 2. Capture every visit in EVV.** Each personal-care, respite, or companion visit must be recorded and verified in the EVV system.
- 3. Submit the claim to the member's MCO.** Using your NPI and the authorized service; the MCO — not the state — pays you. Match each claim to its verified EVV visit.

4. Watch timely filing and reconcile remittances. Confirm each MCO's filing window, and reconcile every remittance so no earned hour goes uncollected.

8.4 Veterans

- **VA Homemaker / Home Health Aide & Veteran-Directed Care:** the VA's in-home programs — contract through the VA Community Care Network. Virginia is in Region 1, administered by Optum (844-839-6120).
- **Aid & Attendance:** an increased VA pension the veteran can apply toward your private-pay care (verify current amounts on VA.gov).

PART 9

What It Costs & How Long It Takes

In short: Budget the license, nurse coverage, insurance & bonding, aide training, EVV, payroll float, and (optionally) accreditation. Launch on private pay, then add Medicaid.

9.1 Startup line items (typical — verify)

Item	What's in it	Ballpark
VDH HCO license	Initial Home Care Organization license	\$500
Nurse coverage	RN/LPN available during personal-care hours + supervision	Ongoing
Business setup	SCC LLC \$100 + \$50/yr registration, EIN free, NPI free, BPOL, legal	~\$150 + local + legal
Insurance & bond	Liability + abuse + non-owned auto + dishonesty bond + workers' comp	Get quotes — annual
Aide training	PCA training pathway per aide + 12 hrs annual	Per aide
EVV & software	EVV + scheduling/billing/payroll platform	Monthly SaaS
Payroll float	You pay aides before Medicaid/insurers pay you	Working capital
Accreditation (optional)	ACHC / CHAP / Joint Commission	~\$7,000–\$14,500 / 3-yr cycle

The cash-flow trap to plan for. You pay aides weekly, but Medicaid and insurers pay weeks later. Private-pay clients smooth this out — line up a business line of credit before you scale Medicaid hours.

9.2 Timeline

Weeks	Entity, nurse & policies	Form the LLC, line up your nurse, write your P&P to 12VAC5-381
Weeks	Insurance & BPOL	Bind coverage & bond; get your local business license
~60 days	VDH application → survey → license	Application + \$500 → pre-licensure conference → on-site survey → license
Right away	Start private pay	Serve private-pay & LTC-insurance clients
Parallel	Enroll for Medicaid	DMAS/Gainwell + Cardinal Care MCO contracts + EVV
Ongoing	Referrals & census	Discharge planners, AAAs, No Wrong Door

9.3 The money math — a simple break-even

Home-care economics are simple once you see them laid out. On each private-pay hour, you bill the client a rate; out of that you pay the caregiver's wage plus payroll taxes, workers' comp, and your overhead. The gap is your gross margin — often roughly 30–40% of the bill rate for a well-run private-pay agency. A worked example, using round numbers you'll replace with your own:

- **Bill rate to the client:** say \$30/hour (set yours from your local market and what referral sources see).
- **Fully-loaded caregiver cost:** the wage (say \$15) plus about 15–20% for payroll taxes, workers' comp, and paid time — roughly \$18/hour all-in.
- **Gross margin:** about \$12/hour, before overhead — from which you cover insurance, software, office, marketing, and your own pay.
- **What this means for you:** you need enough billed hours each week to cover your fixed overhead before you earn a profit — which is why census (total authorized client hours) and caregiver utilization are the two numbers to watch, and why a few reliable, long-hours clients are worth more than many tiny visits.

9.4 The hidden costs new owners miss

The line items above are the visible costs. The ones that sink underprepared owners are the invisible ones — plan for these before you open:

- **Payroll float:** you pay aides every week or two, but the Cardinal Care MCOs and long-term-care insurers pay you weeks after you bill. On a growing Medicaid caseload you can be out tens of thousands of dollars in wages you've paid but not yet collected. Keep 8–12 weeks of payroll in reserve, or launch on private pay (which pays up front) while your Medicaid line ramps.
- **Unbillable overhead:** your own time, your scheduler/coordinator, office and phone, background checks, and the RN/LPN supervisory visits Virginia requires during personal-care hours — none of it is billable, all of it is real.
- **Turnover churn:** every aide who quits costs you re-hiring, re-screening (a new VSP report and sworn disclosure), and re-training. Budget for it, and spend on retention instead — it's cheaper than replacement.
- **Denied and delayed claims:** a Medicaid claim that doesn't match a verified EVV visit, or misses the MCO's timely-filing window, simply doesn't get paid. Reconcile every remittance and chase exceptions weekly.
- **Nurse coverage:** Virginia requires an RN or LPN available during all personal-care hours plus supervisory visits at least every 90 days — an ongoing cost that's easy to underestimate when you model only aide wages.

PART 10

Getting Your First Clients

In short: Clients come from discharge planners, home-health/hospice partners, the 25 Area Agencies on Aging, No Wrong Door Virginia, care managers, and senior communities. Build these before you open.

10.1 The referral sources

- **Hospital discharge planners & case managers:** the highest-volume source for post-hospital home support — make same-day starts easy.
- **Home-health & hospice partners:** they refer the non-skilled hours their benefit doesn't cover; build reciprocal partnerships.
- **Area Agencies on Aging & No Wrong Door:** Virginia has 25 AAAs (vaaaa.org) and the statewide No Wrong Door network powered by VirginiaNavigator — get listed and known; families self-refer there.
- **Cardinal Care MCO care coordinators:** once contracted, they route member hours to in-network agencies.
- **Geriatric care managers & senior communities:** Aging Life Care professionals and assisted-/independent-living communities refer supplemental one-on-one care.
- **Virginia 2-1-1:** list your services with Virginia 2-1-1 (dial 211).

10.2 What to ask each referral source

"How do you decide which home-care agency to refer a family to?"

"What do you need from me to be on your referral or preferred-provider list — license, insurance, bonding, Medicaid enrollment, availability?"

"Who exactly is the discharge planner, social worker, or case manager I should build a relationship with?"

"How fast do you need us to be able to start care after a referral — same day, 24 hours, 48 hours?"

"What makes an agency easy to work with, and what gets one dropped from your list?"

Speed and reliability win home-care referrals more than anything else: be the agency that answers the phone on the first call, can start care within 24–48 hours, communicates back to the referrer, and never no-shows a shift. One reliable start earns you the next ten referrals.

10.3 Your first-90-days referral plan

Referrals don't happen by accident — they come from a handful of relationships you build on purpose. Here's a simple plan for your first 90 days:

1. **Make a target list.** Write down the 10–15 hospital discharge planners, skilled home-health and hospice liaisons, senior-community directors, and case managers in your service area — these are the people who send families to agencies.
2. **Show up in person with a one-pager.** A clean one-page sheet: who you are, what you do, your service area, how fast you can start, your insurance/credentials, and your phone number. Leave it; then follow up a week later.
3. **Be absurdly responsive.** Answer every call, say yes to same-day starts whenever you can, and report back to the referrer after the first visit — that single habit earns the next referral.
4. **Ask for feedback and the next referral.** After a good start, ask the referrer what would make you easier to work with, and whether they have another family who needs help. Then do it again next week.

PART 11

Who to Call & Exactly What to Ask — Every Verify, in One Place

In short: Your master contact + verification hub. Everywhere the guide says “verify,” the who and the what-to-ask are collected here.

Licensing, the administrator & staff

Who	Contact	What to verify / ask
VDH — Office of Licensure & Certification	(804) 367-2102 · vdh.virginia.gov/licensure-and-certification	The HCO application & \$500 fee; the pre-licensure survey; the companion-only exemption
Virginia State Police	vsp.virginia.gov	The criminal record report and sworn disclosure; barrier crimes
Aide training resources	Board of Nursing programs / the PCA curriculum	The 12VAC5-381-290 pathways; the DMAS 40-hour rule if billing Medicaid
Locality — business license (BPOL)	your city/county	Your local business license

Business, insurance & billing IDs

Who	Contact	What to verify / ask
Virginia State Corporation Commission	cis.scc.virginia.gov	Forming your LLC (\$100); the \$50 annual registration
IRS	irs.gov (EIN Assistant)	Get your EIN (free)
NPPEs / CMS	npes.cms.hhs.gov	Your Type 2 NPI (for Medicaid/VA billing)
Insurance agent (home-care program)	trustedchoice.com ; carriers in Part 3.3	Liability + abuse + non-owned auto + dishonesty bond + workers' comp

Funding, EVV & referrals

Who	Contact	What to verify / ask
DMAS — provider enrollment (Gainwell)	vamedicaid.dmas.virginia.gov	Enrolling your NPI; the July 2025 requirements
Cardinal Care MCOs	Aetna, Anthem HealthKeepers, Humana, Sentara, UnitedHealthcare	Contracting; the current personal-care rates; authorizations; timely filing
DMAS — EVV	EVV@dmas.virginia.gov	EVV onboarding and compliance
VA Community Care / Optum (Region 1)	844-839-6120 · va.gov/communitycare	The VA Homemaker/HHA program; joining the CCN

Referral & senior-services partners

Who	Contact	Role
Area Agencies on Aging (25)	vaaaa.org	Local information & referral for older adults
No Wrong Door Virginia / VirginiaNavigator	dars.virginia.gov · virginiannavigator.org	The statewide referral network — get listed
Virginia 2-1-1	dial 211 · 211virginia.org	Free statewide referral line — list your services
Accreditor (optional) — ACHC/CHAP/TJC	achc.org · chapinc.org · jointcommission.org	Whether/when to accredit; MCO/VA requirements

Common questions, answered

Do I have to take Medicaid?

No. Many successful agencies run entirely on private pay, long-term-care insurance, and veterans' benefits — it's often the smarter way to start, because you earn immediately without the enrollment, contracting, and EVV that Medicaid requires. You can add Medicaid later once you have cash flow and a team.

Can I run this from home at first?

Often yes for a small non-medical agency — the care happens in the client's home, so you may not need a storefront to start. Confirm your local zoning and your license/registration rules (some require a business address), and know you'll want real office space as you grow.

Should my caregivers be employees or independent contractors?

Almost always W-2 employees. Classifying caregivers as 1099 contractors to save on taxes is one of the most common — and most expensive — mistakes in home care; it invites wage-and-hour and tax liability, and most payers and insurers require employees. Treat them as employees and carry workers' comp.

How much cash do I really need to start?

Enough to cover several weeks — ideally a couple of months — of caregiver payroll before your first payer reimbursements arrive, plus your license/insurance/software setup. Under-capitalization is the number-one reason new agencies fail; line up a business line of credit before you scale.

How long until it's profitable?

Most agencies take several months to a year to build a steady client base and referral flow. Starting on private pay shortens the runway because you're paid quickly; Medicaid volume comes later and pays slower. Plan your finances for a ramp, not an overnight full census.

Do I need experience in health care?

It helps, but plenty of successful owners come from business, sales, or family-caregiving backgrounds and hire the clinical expertise they need (for example, a nurse where the state requires one). What matters more is being organized, responsive, good with people, and disciplined about compliance and cash.

What actually makes one agency win over another?

Reliability. Families and referral sources choose — and stay with — the agency that answers the phone, starts care fast, sends the same dependable caregiver, communicates, and never leaves a shift unstaffed. Get that right and referrals compound; get it wrong and no amount of marketing saves you.

Do I have to get the VDH license, or can I skip it?

If you provide any hands-on personal care — bathing, dressing, transferring, toileting, or feeding — you must be a licensed Home Care Organization. Only a pure companion, sitter, homemaker, or chore business with zero hands-on care is exempt under 12VAC5-381-30. Most agencies that want a real elder-care market, or to bill Medicaid, need the license.

How long does the VDH license really take?

Plan for roughly two months from a complete application through the pre-licensure conference and on-site survey to your license — plus the weeks it takes to write your policies to the 12VAC5-381 standard first. Start your nurse coverage, insurance, and aide hiring in parallel so you're ready to serve private-pay clients the day the license issues.

Do I need a nurse if I'm only doing personal care?

Yes. Virginia requires an RN or LPN to be available during all hours personal-care services are provided, with supervisory visits at least every 90 days. You can employ the nurse or contract for the coverage, but you must have it in place before you deliver personal care — line it up early, because it's both a licensure requirement and an ongoing cost.

You can do this

Opening a Virginia home care agency is very doable — a single license, a real Cardinal Care market alongside private pay, and a clear companion-only exemption if you want to start simpler. Take it one rung at a time: decide personal-care vs. companion-only, form the business, line up your nurse coverage, get the VDH license, recruit and screen great aides, launch on private pay, add Medicaid through DMAS and the MCOs, and build referral relationships. Verify each fee and contact as you go, and lean on the people in Part 11.

Reminder. Prepared for PolicyReady.org and grounded in Code of Virginia Title 32.1, 12VAC5-381, and current Cardinal Care information as of 2026. This is general information, not legal, tax, or billing advice. Requirements, fees, rates, and contacts change — verify each with VDH, DMAS, and qualified counsel before you rely on it. Confirm the current license fee, the personal-care rates, and your EVV setup before you rely on them.